

PARTNERSHIP PROSPECTUS

THE CARE GOVERNANCE DEFICIT

Transforming Social Policy into Auditable Care Metrics in Low- and Middle-Income Countries (LMICs)

Prepared by:

CareBridge’07 Dynamic Global Ltd
RC: 8734346 | Nigeria

Author:

Ayomide Idris Sanni, MSW
Founder & Managing Director, CareBridge’07 Dynamic Global Ltd

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1. EXECUTIVE SUMMARY

Social protection systems across LMICs face unprecedented strain from accelerating population ageing, urban migration, weakening traditional care networks, and underprepared public institutions. This creates a *Care Governance Deficit* — institutions lack mechanisms to:

- Measure dignity-centered care outcomes
- Verify service quality and accountability
- Monitor caregiver burden and resilience
- Detect abuse, neglect, and social isolation
- Allocate resources efficiently and equitably
- Generate actionable, real-time policy intelligence

Core Principle: What cannot be measured cannot be effectively governed.

CareBridge’07 introduces the *Care Governance Architecture (CGA)* — an evidence-based framework transforming social protection from charity-driven responses into measurable, accountable systems. At its core is the *Geriatric Dignity Index (GDI)*, evaluating care across five domains: Structural Autonomy, Preserved Voice, Psychosocial Legacy, Protection, and Communitarian Participation.

2. THE PROBLEM: CARE GOVERNANCE DEFICIT

Challenge	Impact
Accelerating population ageing	Millions lack adequate care infrastructure
Urban migration separating families	Traditional informal care networks weakening
Declining household sizes	Rising dependency ratios expose care model

Challenge	Impact
	limits
Inadequate public institutions	No tools to measure or verify care outcomes
Reliance on unpaid family labor	Care model unsustainable without state investment

Result: Human dignity remains aspirational rather than operational. Resource allocation and policy reform lack evidence.

3. THE SOLUTION: CARE GOVERNANCE ARCHITECTURE (CGA)

3.1 Core Framework

The CGA enables institutions to: **Assess** vulnerability, **Monitor** care quality, **Strengthen** accountability, **Predict** systemic risks, and **Generate** policy intelligence.

3.2 Central Metric: Geriatric Dignity Index (GDI)

Domain	What It Measures
1. Structural Autonomy	Control over daily routines, preferences, environment
2. Preserved Voice & Executive Agency	Decision-making, financial autonomy, legal rights
3. Psychosocial Legacy & Identity	Social connectedness, cultural continuity, memory
4. Protection & Fundamental Security	Safeguarding from abuse/neglect, physical safety
5. Communitarian Participation	Community integration, intergenerational relationships

These dimensions convert human rights into *administrative indicators* for budgets, audits, M&E systems, and financing.

3.3 Scalable Ecosystem of Indices

Index	Focus Area
GDI	Dignity outcomes for older persons &

Index	Focus Area
	institutional care
DAI	Educational safety, anti-bullying, child safeguarding
DII	Accessibility & inclusion for persons with disabilities
CRI	Economic/psychological resilience of caregivers
CPI	Social vulnerability, displacement risks, community protection

Technology: Mobile data collection, digital dashboards, GIS vulnerability mapping, predictive analytics, AI-assisted decision support.

4. TARGET STAKEHOLDERS & APPLICATIONS

4.1 Governments

National/State/Local Governments, Municipal Authorities: Policy reform, resource allocation
Ministries: Health, Social Welfare, Education, Senior Citizens, Child Protection Agencies

4.2 Development Partners

World Bank, AfDB: Social protection financing, infrastructure development
UNICEF, UNDP: Child safeguarding, governance metrics, public admin reform
International NGOs, Bilateral Agencies: Field implementation, capacity building

4.3 Academic & Research Institutions

Universities, Policy Schools: Research validation, curriculum integration
Gerontology Centres, Social Work/Public Health Faculties: Scientific validation, caregiver training

4.4 Service Providers

CBOs, FBOs, Home-Care Agencies, Social Enterprises, Humanitarian Orgs: Local assessment, quality monitoring, scalable delivery

5. CENTRAL ARGUMENT: FROM CHARITY TO

ACCOUNTABILITY

Human dignity must become an auditable governance asset — measurable, verifiable, financiaible, monitorable, and defensible.

Transformation	Outcome
Compassion → Systems	Structured care delivery
Rights → Metrics	Quantifiable dignity indicators
Care → Measurable Outcomes	Evidence-based evaluation
Vulnerability → Intelligence	Predictive risk management
Policy Intentions → Results	Accountability mechanisms

The CGA transitions LMICs:
From Charity → Accountability | Assumptions → Evidence | Fragmented → Integrated |
Reactive → Predictive | Invisible → Measurable

6. PARTNERSHIP OPPORTUNITIES

Partner Type	Opportunity
Governments	National CGA deployment, M&E integration, budget frameworks
Development Partners	Pilot funding, technical collaboration, scale-up investment
Academia	Research validation, data sharing, joint publications
Service Providers	Implementation training, tech access, quality certification

7. IMPLEMENTATION ROADMAP

Phase	Timeline	Key Activities
Phase 1: Pilot	6–12 months	Country pilots, index validation, dashboard deployment
Phase 2: Scale	12–24 months	Regional expansion, ministry integration, capacity building

Phase	Timeline	Key Activities
Phase 3: Institutionalize	24–36 months	National policy adoption, financing mechanisms

8. ABOUT CAREBRIDGE'07 DYNAMIC GLOBAL LTD

CareBridge'07 Dynamic Global Ltd (RC: 8734346) is a Nigerian-based social innovation and GovTech enterprise transforming human vulnerability into measurable governance intelligence.

Mission: Convert compassion into systems, rights into metrics, and care into measurable outcomes through evidence-based policy.

Leadership: Ayomide Idris Sanni, MSW — Founder & Managing Director. YALI RLC West Africa Alumnus, NG-CARES Program Specialist, Author of *The Dignity of Care*.

9. CONTACT

CareBridge'07 Dynamic Global Ltd

Email: carebridge07global@gmail.com

Nigeria: +234 802 850 7370 | Oman: +968 78538712

Website: <https://carebridge07global-boop.github.io/07dynamic-global/>

LinkedIn: <http://linkedin.com/in/ayomide-sanni-971840103>

"What cannot be measured cannot be effectively governed."

The Care Governance Deficit is a practical blueprint for the next generation of social governance in LMICs.

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